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Time taken	12 secs
Marks	0/70
Grade	0 out of 100

Question 1

Not answered

Marked out of 1

Of the following parasomnias, which one is more common in females than males?

Select one:

- ☐ Sleepwalking
- ☐ REM sleep behavioural disorder
- ☐ Confusional arousals
- ☐ Sleep terrors
- ☐ Nightmares

Your answer is incorrect.

Explanation:

No gender difference has been reported in sleepwalking, sleep terrors, or confusional arousals. Nightmares are reported to be more common in females. REM sleep behaviour disorder is more common in males, particularly those over the age of 50 years.

Ref: Singh S et al. Parasomnias: a comprehensive review. Cureus. 2018 Dec;10(12).

The correct answer is: Nightmares

Question 2

Not answered

Marked out of 1

On corneal examination, Kayser-Fleischer rings are often present in patients with

Select one:

- ☐ Fahr disease
- ☐ Herpes simplex encephalitis
- ☐ Wilson disease
- ☐ Huntington disease
- ☐ Meige syndrome

Your answer is incorrect.

Explanation:

Wilson disease is a genetic disorder of copper metabolism that leads to copper accumulation in various organs, primarily the liver and brain. The clinical symptoms are a result of organ dysfunction due to the direct or indirect effects of copper accumulation. Wilson disease is typically described as a hepato-neurological disease, with focus on signs of liver disease and extrapyramidal symptoms (e.g. wing-beating tremor and dystonia). Copper accumulates in multiple organs over the course of the disease, leading to a variety of clinical manifestations, including neurologic (40-50% at diagnosis), psychiatric (10-25% at diagnosis, and up to 100% over the disease course), ophthalmologic (Kayser-Fleischer rings and sunflower cataracts) and occasionally endocrinologic, cardiologic, or/or osteoskeletal. Kayser-Fleischer rings are green-blue corneal deposits.

Ref: Litwin T et al. Psychiatric manifestations in Wilson's disease: possibilities and difficulties for treatment. Therapeutic advances in psychopharmacology. 2018 Jul;8(7):199-211.

Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 166.

The correct answer is: Wilson disease

Question 3

Not answered

Marked out of 1

Which of the following is preserved in associative visual agnosia?

Select one:

- ☐ Naming objects
- ☐ Drawing objects
- ☐ Using objects
- ☐ All of the listed options
- ☐ None of the listed options

Your answer is incorrect.

Explanation:

Neurologic conditions such as strokes and other focal lesions have facilitated the definition of several disorders of visual perception. Apperceptive visual agnosia is the inability to identify and draw items using visual cues, with preservation of other sensory modalities. It represents failure of transmission of information from the higher visual sensory pathway to the association areas and is caused by bilateral lesions in the visual association areas. Associative visual agnosia is the inability to name or use objects despite the ability to draw them. It is caused by bilateral medial occipitotemporal lesions and can occur along with other visual impairments.

Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 927.

The correct answer is: Drawing objects

Question **4**

Not answered

Marked out of 1

Which of the following tremors is amplified as a target is reached?

Select one:

- ☐ Parkinsonism
- ☐ Resting tremor
- ☐ Drug-induced tremor
- ☐ Myoclonic jerk
- ☐ Cerebellar tremor

Your answer is incorrect.

Explanation:



Tremor type	Description
RESTING	Occurs in a body part that is relaxed and completely supported against gravity Most commonly caused by parkinsonism , but may also occur in severe essential tremor
ACTION	Occurs with voluntary contraction of muscle Includes postural, isometric, and kinetic tremors
Postural	Occurs when the body part is voluntarily maintained against gravity Includes essential, physiologic, cerebellar, dystonic, and drug-induced tremors
Isometric	Occurs as a result of muscle contraction against a rigid stationary object
Kinetic	Occurs with any form of voluntary movement Includes classic cerebellar, dystonic, and drug-induced tremors Essential tremor can cross over to this category
Intention	Subtype of kinetic tremor amplified as the target is reached Presence of this type of tremor implies that there is a disturbance of the cerebellum or its pathways

Adapted from Crawford and Zimmerman 2018

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Ref: Crawford III PF, Zimmerman EE. Tremor: Sorting through the differential diagnosis. American family physician. 2018 Feb 1;97(3):180-6.

The correct answer is: Cerebellar tremor



Question 5

Not answered

Marked out of 1

A 76-year-old man is brought to the emergency unit by his wife. He had breakfast that morning, but appeared unable to remember that he and his wife had planned a day out. He repeatedly enquired where she was taking him. She reports that he appeared bewildered, but was alert and had no sensory and motor deficits throughout. He returned to a normal mental state after 10 hours. What is the most likely diagnosis?

Select one:

- ☐ Parkinson disease
- ☐ Alzheimer disease
- ☐ Transient global amnesia
- ☐ Creutzfeldt-Jakob disease
- ☐ Normal pressure hydrocephalus

Your answer is incorrect.

Explanation:

The fundamental clinical feature of transient global amnesia (TGA) is the acute onset of amnesia. During an attack, patients cannot memorise or learn new information, that is, they have anterograde amnesia. They also cannot recall information acquired very recently, such as the events in the last several hours or days (i.e., they also have retrograde amnesia). Most patients seem calm but bewildered. TGA lasts for 3 to 24 hours but is most severe during the initial 1-2 hours. By definition, the total duration must not exceed 24 hours.

Ref: Kaufman DM, Geyer HL, Milstein MJ, Rosengard JL. Kaufman's Clinical Neurology for Psychiatrists, 9th Edition. 2023. p. 113.

TRANSIENT GLOBAL AMNESIA: Diagnostic criteria (Hodges and Warlow)

Attacks must be witnessed, and information must be available from a capable observer who was present for most of the attack

There must be clear-cut anterograde amnesia during the attack

Clouding of consciousness and loss of personal identity must be absent, and the cognitive impairment must be limited to amnesia (e.g., no aphasia, apraxia)

There should be no accompanying focal neurologic symptoms during the attack and no significant neurologic signs afterwards

Epileptic features must be absent

Attacks must resolve within 24 hr

Patients with recent head injury or active epilepsy (i.e., currently on medication or at least 1 seizure in the past 2 years) are excluded

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The correct answer is: Transient global amnesia



Question 6

Not answered

Marked out of 1

Which of the following drugs is contraindicated in angle closure glaucoma?

Select one:

- ☐ Topiramate
- ☐ Clonazepam
- ☐ Lithium
- ☐ Valproate
- ☐ Carbamazepine

Your answer is incorrect.

Explanation:

Angle closure is the form of glaucoma that can be induced or worsened by psychotropic medication. Topiramate has been associated with the development of closed-angle glaucoma, potentially due to swelling of the lens and ciliary body.

Ref: Taylor DM, Gaughran F, Pillinger T. The Maudsley Practice Guidelines for Physical Health Conditions in Psychiatry. 2021. p. 520.

The correct answer is: Topiramate

Question 7

Not answered

Marked out of 1

Which of the following drugs has the best available evidence for the treatment of depression following a myocardial infarction?

Select one:

- ☐ Venlafaxine
- ☐ Doxepin
- ☐ Mirtazapine
- ☐ Amitriptyline
- ☐ Sertraline

Your answer is incorrect.

Explanation:

Antidepressants are best avoided in the first 2 months following a myocardial infarction. If clinically indicated, SSRIs (sertraline is the drug of choice; avoid fluvoxamine and citalopram/escitalopram) should be prescribed rather than TCAs. If sedation is required, consider use of mirtazapine or a small dose of trazodone at night.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 1033.

The correct answer is: Sertraline

Question 8

Not answered

Marked out of 1

What is the risk of depression in a cardiac patient with congestive heart failure compared to the general population?

Select one:

- ☐ 3:1
- ☐ 1:8
- ☐ 5:1
- ☐ 1:7
- ☐ 1:5

Your answer is incorrect.

Explanation:

Clinically significant depressive symptoms affect 21.5% of heart failure (HF) patients, with one-third of HF patients reporting elevated depressive symptoms on questionnaires and 19% meeting criteria for a depressive disorder (typically major depressive disorder, but some studies have included dysthymia and minor depression). These prevalence rates are similar in studies of inpatients and outpatients, and they are approximately two-to-three times higher than the general population.

Ref: Celano CM et al. Depression and anxiety in heart failure: a review. Harvard review of psychiatry. 2018 Jul;26(4):175.

The correct answer is: 3:1



Question 9

Not answered

Marked out of 1

The mean age of onset of chronic fatigue syndrome is between

Select one:

- ☐ 29-35 years
- ☐ 15-25 years
- ☐ 10-14 years
- ☐ 46-55 years
- ☐ 36-45 years

Your answer is incorrect.

Explanation:

Chronic fatigue syndrome (CFS) predominantly affects adults, with a peak age of onset between 20 and 45 years, and a female to male ratio of 3:1. The average age of onset is 33 years, but CFS can develop in people as old as 77 years. Patients can be ill for years or even decades. They most commonly report a fluctuating illness pattern in which symptoms wax and wane but are always present.

Ref: Cortes Rivera M et al. Myalgic encephalomyelitis/chronic fatigue syndrome: a comprehensive review. *Diagnostics*. 2019 Sep;9(3):91.

Bateman L et al. Myalgic encephalomyelitis/chronic fatigue syndrome: essentials of diagnosis and management. In *Mayo Clinic Proceedings* 2021 Nov 1 (Vol. 96, No. 11, pp. 2861-2878).

The correct answer is: 29-35 years

Question 10

Not answered

Marked out of 1

The ratio of prevalence of chronic fatigue syndrome in males compared to females is

Select one:

- ☐ 3:1
- ☐ 2:1
- ☐ 1:1
- ☐ 1:6
- ☐ 1:3

Your answer is incorrect.

Explanation:

Chronic fatigue syndrome (CFS) predominantly affects adults, with a peak age of onset between 20 and 45 years, and a male to female ratio of 1:3. The average age of onset is 33 years, but CFS can develop in people as old as 77 years. Patients can be ill for years or even decades. They most commonly report a fluctuating illness pattern in which symptoms wax and wane but are always present.

Ref: Cortes Rivera M et al. Myalgic encephalomyelitis/chronic fatigue syndrome: a comprehensive review. *Diagnostics*. 2019 Sep;9(3):91.

Bateman L et al. Myalgic encephalomyelitis/chronic fatigue syndrome: essentials of diagnosis and management. In *Mayo Clinic Proceedings* 2021 Nov 1 (Vol. 96, No. 11, pp. 2861-2878).

The correct answer is: 1:3

Question 11

Not answered

Marked out of 1

Diagnostic features of chronic fatigue syndrome include all of the following except:

Select one:

- ☐ Muscular aches and pains
- ☐ Unrefreshing sleep
- ☐ Duration >12 months
- ☐ Post-exertional malaise
- ☐ Lack of energy

Your answer is incorrect.

Myalgic encephalomyelitis/chronic fatigue syndrome (ME/CFS) is a debilitating disease with core symptoms of fatigue, unrefreshing sleep, post-exertional malaise, and cognitive dysfunction. The aetiology of the illness has yet to be revealed, which has led to no established objective diagnostics, pathophysiology, or therapeutics. Accordingly, many expert groups have developed case definitions based on clinical features. To date, over 20 ME/CFS case definitions have been established by different groups in various countries. The most widely used have been the Fukuda criteria and the international criteria.

A case of ME/CFS is defined by the presence of an unexplained, persistent, and relapsing chronic fatigue of new onset that is not alleviated by rest, which results in a significant reduction in the quality of life, and a concurrent occurrence of four or more of the following eight symptoms that must have persisted during six or more consecutive months:

- 1) 'Brain fog' – described as impairment in short-term memory or concentration severe enough to cause a reduction in previous levels of personal activities
- 2) Sore throat
- 3) Tender cervical or axillary nodes
- 4) Muscle pain
- 5) Multipoint pain without joint swelling

- 6) Headaches
- 7) Unrefreshing sleep
- 8) Post-exertional malaise lasting more than 24 hours

Ref: Lim EJ, Son CG. Review of case definitions for myalgic encephalomyelitis/chronic fatigue syndrome (ME/CFS). Journal of translational medicine. 2020 Dec;18(1):1-10.

Cortes Rivera M et al. Myalgic encephalomyelitis/chronic fatigue syndrome: a comprehensive review. Diagnostics. 2019 Sep;9(3):91.

The correct answer is: Duration >12 months



Question 12

Not answered

Marked out of 1

A 42-year-old woman has a history of 2 episodes of unilateral blindness that lasted for few weeks. She now presents with paraesthesia of the left leg. An MRI shows multiple demyelinating plaques. Which of the following is correct with respect to her illness?

Select one:

- ☐ It has a uniform, progressive course
- ☐ It is due to a bacterial infection
- ☐ It presents with diverse physical signs
- ☐ It is more common in men than women
- ☐ It occurs more commonly near the equator

Your answer is incorrect.

Explanation:

Multiple sclerosis (MS) is an immune-mediated demyelinating disease. The cause is uncertain, but likely due to environmental factors (e.g. Epstein-Barr virus infection, vitamin D deficiency, latitude, and smoking) interacting with genetic predisposition. MS is twice as common in women as in men. It is seen with greater frequency as the distance from the equator increases (i.e. in White populations living in northern latitudes). It is characterized by multiple demyelinating lesions with a predilection for the optic nerves, cerebellum, brainstem, and spinal cord. MS is predominantly a white matter disease. Diverse physical signs reflect the presence and distribution of plaques. 85-90% of cases start with episode of acute neurological symptoms (e.g. optic neuritis). Most patients follow a relapsing-remitting course.

Ref: Ron MA. The neuropsychiatry of multiple sclerosis. BJPsych Advances. 2018 May;24(3):178-87.

The correct answer is: It presents with diverse physical signs

Question 13

Not answered

Marked out of 1

Which of the following is not characteristic of chronic fatigue syndrome?

Select one:

- ☐ Significant weight loss
- ☐ Impaired occupational function
- ☐ Sore throat
- ☐ Joint pains
- ☐ Cervical lymphadenopathy

Your answer is incorrect.

Explanation:

Chronic fatigue syndrome is a disabling clinical condition characterised by unexplained and persistent post-exertional fatigue accompanied by a variety of symptoms related to cognitive, immunological, endocrinological, and autonomic dysfunction. It is also characterised by a substantial reduction in previous levels of occupational, educational, social, and personal activities. Unemployment rates among those who suffer from CFS are between 35-69%.

Criteria	Definition of Chronic Fatigue Syndrome (US Center for Disease Control and Prevention, 1994)
Main criteria	Persistent or relapsing fatigue of 6 months duration or more Fatigue being not explained by any concurrent somatic or psychiatric condition Fatigue is new or has a defined onset Fatigue is not due to ongoing exertion Fatigue is not relieved by rest
Additional criteria (at least 4)	Impaired memory and/or concentration that causes a reduction in previous level of activity Sore throat Tender cervical and/or axillary lymph nodes Muscle pain Multi-joint pain New headaches Unrefreshing sleep Post-exertional malaise
Exclusion criteria	Medical condition explaining fatigue (e.g. untreated hypothyroidism, sleep apnoea, narcolepsy) Previously diagnosed medical condition whose resolution has not been documented Past or current diagnosis of MDD or bipolar disorder, schizophrenia, dementia, delusional disorder, anorexia nervosa, bulimia nervosa, substance use disorder within 2 years before onset Severe obesity (BMI > 45) Unexplained physical exam/lab/imaging abnormality that strongly suggests exclusionary condition

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Ref: Cortes Rivera M et al. Myalgic encephalomyelitis/chronic fatigue syndrome: a comprehensive review. Diagnostics. 2019 Sep;9(3):91.

The correct answer is: Significant weight loss



Question 14

Not answered

Marked out of 1

Which of the following is a feature of sleep terrors?

Select one:

- ☐ Vivid memory of dreams on waking up
- ☐ Acting out
- ☐ Occurring in REM sleep
- ☐ Increased muscle tone
- ☐ Average duration of 1-2 minutes

Your answer is incorrect.

Explanation:

Sleep terrors are recurrent episodes of abrupt awakening from sleep characterised by a panicky scream, with intense fear and autonomic arousal. The individual usually has no recall of the details of the event and is unresponsive to stimuli (e.g conversation) during the episode. Sleep terrors typically occur during the first third of the night, during slow-wave stage 3 of NREM sleep. Autonomic hyperactivity is manifested by tachycardia, tachypnea, diaphoresis, flushed face, dilated pupils, agitation, tremulousness, and increased muscle tone. Episodes usually last for 5-10 minutes.

Ref: Leung AK et al. Sleep terrors: an updated review. Current pediatric reviews. 2020 Aug 1;16(3):176-82.

Sample D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 462.

The correct answer is: Increased muscle tone

Question 15

Not answered

Marked out of 1

Which of the following is an example of subcortical dementia?

Select one:

- ☐ Creutzfeldt-Jakob disease
- ☐ Parkinson disease dementia
- ☐ Alzheimer disease
- ☐ Lewy body dementia
- ☐ Pick disease

Your answer is incorrect.

Explanation:

Cortical dementias consist of illnesses in which neuropsychologic signs of cortical dysfunction – typically aphasia, agnosia, and apraxia – accompany dementia. Because the brain's subcortical areas are relatively untouched, patients remain alert, attentive, and ambulatory. Alzheimer disease serves as the prime example of a cortical dementia. In contrast, subcortical dementias are typified by affective change, and slowed mental processing, which overshadow cognitive impairment. Gait abnormalities constitute the other core feature. Prime examples are Parkinson disease, NPH, HIV-associated dementia, vascular cognitive impairment, and MS.

Classification of dementias

Cortical	Subcortical	Cortico-subcortical
• Alzheimer disease • Frontotemporal dementia (including "Pick disease")	• Vascular dementia (including Binswanger disease) • Progressive supranuclear palsy • Multiple system atrophy • Huntington disease • Parkinson disease • Wilson disease • Multiple sclerosis • Hydrocephalus • HIV-associated dementia	• Dementia with Lewy bodies • Creutzfeldt-Jacob disease (multi-focal, rapid, and aggressive) • Corticobasal degeneration

Adapted from Shaik and Varma 2012



Ref: Kaufman DM, Geyer HL, Milstein MJ, Rosengard JL. Kaufman's Clinical Neurology for Psychiatrists, 9th Edition. 2023. p. 115.

Shaik SS, Varma AR. Differentiating the dementias: a neurological approach. Progress in Neurology and Psychiatry. 2012 Jan;16(1):11-8.

The correct answer is: Parkinson disease dementia



Question 16

Not answered

Marked out of 1

A 33-year-old woman has been referred by her dentist for a long history of facial tingling, numbness, nausea and temporomandibular joint pain. Her symptoms have been anatomically inconsistent, and the pain has tended to fluctuate. She has insisted on multiple investigations, which have shown no abnormalities. The most likely diagnosis is

Select one:

- ☐ Generalised anxiety disorder
- ☐ Bodily distress disorder
- ☐ Depressive disorder with hypochondriacal delusions
- ☐ Delusional disorder
- ☐ Hypochondriasis

Your answer is incorrect.

Explanation:

In ICD-11, bodily distress disorder has replaced the somatoform disorders of ICD-10. Bodily distress disorder is characterised by the presence of bodily symptoms that are distressing to the individual and excessive attention toward the symptoms, which may be manifest by repeated contact with health care providers. Bodily symptoms are persistent, being present on most days for at least several months. Typically, bodily distress disorder involves multiple bodily symptoms that may vary over time. Occasionally there is a single symptom – usually pain or fatigue – that is associated with the other features of the disorder. The symptoms and associated distress and preoccupation have at least some impact on the individual's functioning.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 867.

International Statistical Classification of Diseases and Related Health Problems (11th ed. ICD-11; World Health Organization, 2020)

The correct answer is: Bodily distress disorder

Question 17

Not answered

Marked out of 1

A cognitive faculty commonly affected in patients with post-concussion syndrome is

Select one:

- ☐ Visual memory
- ☐ Line orientation
- ☐ Digit span
- ☐ Speed of processing
- ☐ Time perception

Your answer is incorrect.

Explanation:

Attention deficits and impairment of processing speed are commonly seen in mild traumatic brain injury and post-concussion syndrome. Situational factors contributing to neuropsychological disability in these cases include rapid processing demands, need for complex cognitive attention, and external distractions.

Ref: Broshek DK et al. A review of post-concussion syndrome and psychological factors associated with concussion. Brain injury. 2015 Jan 28;29(2):228-37.

Kaltainen H et al. Mild traumatic brain injury affects cognitive processing and modifies oscillatory brain activity during attentional tasks. Journal of Neurotrauma. 2019 Jul 15;36(14):2222-32.

The correct answer is: Speed of processing

Question 18

Not answered

Marked out of 1

Which of the following are commonly seen in patients with post-concussion syndrome?

Select one:

- ☐ Significant CT and EEG changes
- ☐ Tinnitus, vertigo, and ataxia
- ☐ Short term memory loss and personality changes
- ☐ Dizziness, headaches, irritability, and slowed information processing
- ☐ Nystagmus, dizziness, and memory loss

Your answer is incorrect.

Explanation:

Post-concussion syndrome is characterised by one or more core symptoms – headache, memory impairment, and insomnia – lasting more than 2 to 3 months after a head injury. These and other symptoms of post-concussion syndrome – including irritability, sleepiness, tinnitus – remain entirely subjective and often nonspecific, variable, and occasionally unending. Patients with post-concussion syndrome characteristically describe mild amnesia accompanied by inattention or easy distractibility, slowed information processing, and difficulty completing complex mental tasks. Post-concussion syndrome patients also often report dizziness. In most cases, the dizziness is not authentic vertigo, but a nonspecific sensation with variable, idiosyncratic meanings that substitutes for lightheadedness, anxiety, unsteadiness, or lassitude. Patients have a normal physical and neurologic examination, CT, and MRI. Although the EEG may show minor abnormalities, the changes are generally inconsistent, insignificant, and often attributable to medications.

Ref: Kaufman DM, Geyer HL, Milstein MJ, Rosengard JL. Kaufman's Clinical Neurology for Psychiatrists, 9th Edition. 2023. pp. 559-560.

The correct answer is: Dizziness, headaches, irritability, and slowed information processing

Question 19

Not answered

Marked out of 1

A geriatric patient had a stroke 8 months ago. He now presents with retardation, amotivation, and anhedonia. He does not have any tearfulness or guilt. His MMSE score is 22/30 and he has executive dysfunction. What is his most likely diagnosis?

Select one:

- ☐ Vascular depression
- ☐ Lewy body dementia
- ☐ Alzheimer dementia
- ☐ Psychosis
- ☐ Vascular dementia

Your answer is incorrect.

Explanation:

The vascular depression hypothesis postulates that cerebrovascular disease may predispose, precipitate, or perpetuate some geriatric depressive syndromes. Neurological signs and/or neurological findings, usually executive dysfunction, are found in most patients with vascular depression depending on the location and extent of lesions. Patients with vascular depression often present with retardation, anhedonia, lack of insight into their illness, and disability. They are less likely to report feelings of guilt.

Ref: Alexopoulos GS. Mechanisms and treatment of late-life depression. Translational psychiatry. 2019 Aug 5;9(1):188.

The correct answer is: Vascular depression

Question **20**

Not answered

Marked out of 1

A 29-year-old woman is 6 months pregnant. She is depressed, has a previous history of depression, and a family history of depression. She has responded well to paroxetine in the past. If she were to take paroxetine for the remainder of her pregnancy, which of the following effects is most likely to be seen in the newborn?

Select one:

- ☐ Irritability
- ☐ Neural tube defects
- ☐ Facial abnormalities
- ☐ Ebstein anomaly
- ☐ Neonatal bleeding

Your answer is incorrect.

Explanation:

SSRIs do not appear to be major teratogens, with most data supporting the safety of fluoxetine. When taken late in pregnancy, SSRIs may increase the risk of persistent pulmonary hypertension of the newborn; the absolute risk is very low. Women who take SSRIs may be at increased risk of postpartum haemorrhage. The neonate may experience discontinuation symptoms, which are usually mild (such as agitation or irritability), or rarely respiratory distress and convulsions, with SSRIs. The risk is particularly high with short half-life drugs such as paroxetine and venlafaxine.

Ref: Taylor DM, Barnes TRE, Young AH. The Maudsley Prescribing Guidelines in Psychiatry, 14th Edition. 2021. pp. 685-689.

The correct answer is: Irritability

Question **21**

Not answered

Marked out of 1

An antidepressant with best evidence for use in post stroke depression is

Select one:

- ☐ Nortriptyline
- ☐ desipramine
- ☐ Protriptyline
- ☐ Dosulepin
- ☐ imipramine

Your answer is incorrect.

Nortriptyline is a second-generation tricyclic antidepressant. Patients with cardiovascular disease should be given nortriptyline only under close supervision because of the tendency of the drug to produce tachycardia and to prolong the conduction time (QT) like most other tricyclics.

The correct answer is: Nortriptyline

Question **22**

Not answered

Marked out of 1

Which of the following pregnant mothers has the highest risk of having a child with ADHD?

Select one:

- ☐ 30 year old woman who drinks one glass of red wine a day
- ☐ 31 year old woman who drinks 3 mugs of coffee a day
- ☐ 21 year old woman who smokes 12 cigarettes a day
- ☐ 26 year old woman who has no substance use habits
- ☐ 22 year old woman who smokes one joint of marijuana a day

Your answer is incorrect.

Explanation:

Maternal smoking during pregnancy is associated with an increased risk of ADHD in children. Maternal pre-pregnancy obesity and overweight, pre-eclampsia, hypertension, acetaminophen exposure, and smoking during pregnancy are strongly associated with ADHD.

Ref: He Y et al. Maternal smoking during pregnancy and ADHD: results from a systematic review and meta-analysis of prospective cohort studies. Journal of attention disorders. 2020 Oct;24(12):1637-47.

Kim JH et al. Environmental risk factors, protective factors, and peripheral biomarkers for ADHD: an umbrella review. The Lancet Psychiatry. 2020 Nov 1;7(11):955-70.

The correct answer is: 21 year old woman who smokes 12 cigarettes a day



Question 23

Not answered

Marked out of 1

A 21-year-old woman has been diagnosed with postpartum depression and is currently receiving antidepressant treatment. Which observer rating would be most appropriate to use to assess her response to treatment?

Select one:

- ☐ Major depression inventory
- ☐ Beck depression scale
- ☐ Zung depression scale
- ☐ Edinburgh postnatal depression scale
- ☐ Montgomery Asberg depression rating scale

Your answer is incorrect.

Explanation:

The Montgomery-Asberg Depression Rating Scale (MADRS) is a 10-item observer-rated scale. Each item is rated 0-6, with a total score obtained. Response to an antidepressant is usually defined as a 50% or more decrease from baseline on the MADRS, and remission is defined as a score of 10 or less. The Edinburgh Postnatal Depression Scale is a 10-item tool developed for **screening** postpartum women for depression in outpatient clinics, home visits, or during postpartum appointments.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 97.

Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 603.

Smith-Nielsen J et al. Validation of the Edinburgh Postnatal Depression Scale against both DSM-5 and ICD-10 diagnostic criteria for depression. BMC psychiatry. 2018 Dec;18(1):1-2.

The correct answer is: Montgomery Asberg depression rating scale

Question **24**

Not answered

Marked out of 1

Which of the following is correct with regard to the mechanism of action of modafinil?

Select one:

- ☐ It inhibits glutamatergic transmission
- ☐ It inhibits noradrenaline in the hypothalamus
- ☐ Its half-life is 24 hours
- ☐ It reduces dopamine in the nucleus accumbens
- ☐ It activates neurons that release orexin

Your answer is incorrect.

Explanation:

Exactly how modafinil works is unknown, but it is clearly different from classical stimulants such as methylphenidate and amphetamine. Blockade of the dopamine transporter (DAT) by modafinil leads to increased tonic dopaminergic firing and downstream effects on wake-promoting neurotransmitters. Specifically, cortical release of wake-promoting neurotransmitters is increased, which leads to downstream release of histamine from the tuberomammillary nucleus and further activation of the lateral hypothalamus, with corresponding orexin release that stabilises wakefulness. Modafinil activates hypothalamic neurons that release orexin/hypocretin. The elimination half-life of modafinil is 10-12 hours.

Ref: Stahl SM. Stahl's Essential Psychopharmacology: Prescriber's Guide, 7th Edition. 2021. pp. 521-523.

Stahl SM. Stahl's Essential Psychopharmacology: Neuroscientific Basis and Practical Applications, 5th Edition. 2021. p. 445.

The correct answer is: It activates neurons that release orexin

Question 25

Not answered

Marked out of 1

Modafinil, which is used for daytime sleepiness, causes which of the following neurochemical changes?

Select one:

- ☐ Sustained plasma levels for 24 hours
- ☐ Increased synaptic dopamine
- ☐ Release of serotonin
- ☐ Blockade of orexin release
- ☐ Inhibition of the lateral hypothalamus

Your answer is incorrect.

Explanation:

Increased synaptic dopamine following blockade of the dopamine transporter by modafinil leads to increased tonic firing and downstream effects on neurotransmitters involved in wakefulness. Specifically, this leads to downstream release of histamine from the tuberomammillary nucleus and further activation of the lateral hypothalamus, with corresponding orexin release that stabilises wakefulness. Modafinil activates hypothalamic neurons that release orexin/hypocretin. The elimination half-life of modafinil is 10-12 hours.

Ref: Stahl SM. Stahl's Essential Psychopharmacology: Neuroscientific Basis and Practical Applications, 5th Edition. 2021. pp. 444-445.

Stahl SM. Stahl's Essential Psychopharmacology: Prescriber's Guide, 7th Edition. 2021. pp. 521-523.

The correct answer is: Increased synaptic dopamine

Question **26**

Not answered

Marked out of 1

Which of the following is a common side effect of modafinil?

Select one:

- ☐ Increased appetite
- ☐ Psychosis
- ☐ Headache
- ☐ Depression
- ☐ Weight gain

Your answer is incorrect.

Explanation:

Modafinil most commonly causes headache (dose-dependent). Other notable side effects include anxiety, nervousness, insomnia, dry mouth, diarrhoea, nausea, anorexia, pharyngitis, rhinitis, hypertension, and palpitations. Modafinil can cause CNS effects similar to those caused by other CNS agents (e.g. changes in mood and, theoretically, activation of psychosis, mania, or suicidal ideation).

Ref: Stahl SM. Stahl's Essential Psychopharmacology: Prescriber's Guide, 7th Edition. 2021. pp. 522-523.

The correct answer is: Headache

Question **27**

Not answered

Marked out of 1

Which of the following antidepressants has an effect in common with anti-nausea drugs such as ondansetron?

Select one:

- ☐ Venlafaxine
- ☐ Mirtazapine
- ☐ Duloxetine
- ☐ Bupropion
- ☐ Buspirone

Your answer is incorrect.

Explanation:

Mirtazapine has 5HT₃ antagonism action. The 5HT₃ receptors best known to clinicians are perhaps those localised in the chemoreceptor trigger zone of the brainstem - where they mediate nausea and vomiting, especially in response to cancer chemotherapy - and also those localised in the gastrointestinal tract itself - where they mediate nausea, vomiting, and diarrhoea/bowel motility when stimulated by serotonin. Blocking these 5HT₃ receptors can therefore protect against chemotherapy-induced nausea and vomiting as well as against serotonin-induced gastrointestinal side effects that can accompany agents that increase serotonin. Oral disintegrating tablets of mirtazapine have been used peri-operatively for nausea.

Ref: Stahl SM. Stahl's Essential Psychopharmacology: Neuroscientific Basis and Practical Applications, 5th Edition. 2021. pp. 309-311.

Taylor DM, Barnes TRE, Young AH. The Maudsley Prescribing Guidelines in Psychiatry, 14th Edition. 2021. p. 891.

The correct answer is: Mirtazapine

Question 28

Not answered

Marked out of 1

A 30-year-old woman with a history of bipolar disorder is currently breastfeeding her child. She has a manic relapse, though the presentation is not severe enough to warrant hospitalisation. What would be the best medication to treat her relapse?

Select one:

- ☐ Lithium
- ☐ Lorazepam
- ☐ Lofepramine
- ☐ Chloral Hydrate
- ☐ Sodium Valproate

Your answer is incorrect.

Maudsley prescribing guidelines advises us to refrain from using any mood stabilisers if at all possible in breast feeding mothers. If necessary one may use valproate. Treatment of bipolar disorder invariably requires mood stabilizers, lorazepam and chloral hydrate can be safely excluded. Maternal use of lithium and carbamazepine has been associated with serious difficulties in babies. Lithium increases the risk of thyroid dysfunction, cyanosis, poor muscle tone, and ECG changes in infants. Also renal clearance is decreased in infants up to at least 5 months of age, so the use of lithium during breastfeeding is not advisable unless the mother was already on lithium and discontinuation led to severe relapse not treatable by any other drug. In this case both infant and mother's lithium levels need monitoring. There has been a single recorded case of serious blood abnormalities following exposure to sodium valproate through breast milk, and hepatotoxicity can occur when valproate is directly administered to infants. Therefore, when administering valproate to breastfeeding mothers, pediatric clinical status, liver enzymes, and platelets should be carefully monitored. Carbamazepine can cause hepatic dysfunction and so it is advisable to monitor liver enzymes, bilirubin, and WBC counts frequently and to assay for carbamazepine levels in exposed infants. There is limited, though reassuring data on lamotrigine and no data for the use of gabapentin by nursing mothers. But these agents are not first-line medications for bipolar disorder. Burt et al. (2001) The use of psychotropic medication during breastfeeding. Am J Psychiatry 158:1001-1009.

The correct answer is: Sodium Valproate

Question 29

Not answered

Marked out of 1

Which of the following is a characteristic sign of hepatic encephalopathy?

Select one:

- ☐ Visual hallucinations
- ☐ Asterixis
- ☐ Neck rigidity
- ☐ Tremors
- ☐ Ataxia

Your answer is incorrect.

Explanation:

Patients with hepatic encephalopathy have asterixis. Asterixis can be elicited by having patients extend their arms and hands as though they were stopping traffic. Their hands intermittently quickly fall forward and slowly return, as though flapping or waving good-bye.

Ref: Kaufman DM, Geyer HL, Milstein MJ, Rosengard JL. Kaufman's Clinical Neurology for Psychiatrists, 9th Edition. 2023. pp. 136-137.

The correct answer is: Asterixis

Question **30**

Not answered

Marked out of 1

A 45-year-old woman comes to see you with her husband. He is concerned about her behaviour - she seems on a constant high and has spent a lot of their money on shopping sprees. She is completely unconcerned about that but is worried about a rash on her face and tells you that she has had several mouth ulcers recently. The rash on her face gets worse on sunny days. What is the most likely diagnosis?

Select one:

- ☐ Inhalant (solvent) abuse
- ☐ Cocaine abuse
- ☐ Hyperthyroidism
- ☐ Systemic lupus erythematosus
- ☐ Bipolar disorder

Your answer is incorrect.

It is important to note that SLE can present as camouflaged bipolar disorder.

The correct answer is: Systemic lupus erythematosus



Question **31**

Not answered

Marked out of 1

Which of the following modes of psychotherapy has established evidence in HIV associated depression?

Select one:

- ☐ Dialectic behavioural therapy
- ☐ Interpersonal therapy
- ☐ Mentalisation approach
- ☐ CBT
- ☐ Psychodynamic short term therapy

Your answer is incorrect.

Interpersonal therapy has established evidence in HIV related depression. Markowitz JC, et al. Treatment of depressive symptoms in human immunodeficiency virus-positive patients. Arch Gen Psychiatry. 1998;55:452-457.

The correct answer is: Interpersonal therapy



Question **32**

Not answered

Marked out of 1

A man presents with drooping of the left lip and inability to fully close his left eye though he can frown. What is the likely cause of the lesion?

Select one:

- ☐ Facial nerve palsy (upper motor neuron type)
- ☐ Facial nerve palsy(lower motor neuron type)
- ☐ Trigeminal nerve palsy (motor nucleus)
- ☐ Bulbar palsy
- ☐ Trigeminal nerve palsy (sensory nucleus)

Your answer is incorrect.

In UMN palsy of facial nerve, the upper part of face is spared. This is because the forehead muscles receive input from both ipsilateral and contralateral higher cortical centres.

The correct answer is: Facial nerve palsy (upper motor neuron type)



Question **33**

Not answered

Marked out of 1

In which of the following neurological conditions is anaemia a common finding?

Select one:

- ☐ Friedreich ataxia
- ☐ Brown Sequard syndrome
- ☐ Huntington disease
- ☐ Tabes dorsalis
- ☐ Subacute combined degeneration

Your answer is incorrect.

Explanation:

Subacute combined degeneration is a demyelinated disease commonly associated with vitamin B12 deficiency. It is commonly accompanied by anaemia.

Ref: Cao J, Su ZY, Xu SB, Liu CC. Subacute combined degeneration: a retrospective study of 68 cases with short-term follow-up. European Neurology. 2018;79(5-6):247-55.

The correct answer is: Subacute combined degeneration

Question **34**

Not answered

Marked out of 1

The prevalence of co-morbid depression in those with coronary heart disease is approximately

Select one:

- ☐ 20%
- ☐ 2%
- ☐ 50%
- ☐ 40%
- ☐ 5%

Your answer is incorrect.

Explanation:

The prevalence of depression is significantly higher in patients with coronary heart disease (CHD). More than 20% of patients with CHD are depressed, with the risk of depression highest in those with more severe CHD. CHD patients with depression are at increased risk for recurrent cardiovascular events and mortality.

Ref: De Hert M et al. The intriguing relationship between coronary heart disease and mental disorders. Dialogues in clinical neuroscience. 2018 Mar;20(1):31.

The correct answer is: 20%



Question **35**

Not answered

Marked out of 1

Compared to the general population, suicide rates in patients with Huntington disease are

Select one:

- ☐ 10 times higher
- ☐ Equivalent
- ☐ 4 times higher
- ☐ 20 times higher
- ☐ 2 times higher

Your answer is incorrect.

Explanation:

Suicidal ideation/attempts are common in Huntington disease. Rates of suicide are approximately 4 times higher than the general population. Risk of suicide is correlated with family history of suicide, history of previous attempts, and presence of depression, especially in prodromal stages.

Ref: Bachoud-Lévi AC et al. International Guidelines for the treatment of Huntington's Disease. Frontiers in neurology. 2019;10:710.

Kachian ZR et al. Suicidal ideation and behavior in Huntington's disease: Systematic review and recommendations. Journal of affective disorders. 2019 May 1;250:319-29.

The correct answer is: 4 times higher

Question **36**

Not answered

Marked out of 1

Amnesia is commonly seen in the setting of

Select one:

- ☐ Bacterial meningitis
- ☐ Tuberculous meningitis
- ☐ Cerebral lupus
- ☐ Herpes simplex encephalitis
- ☐ HIV encephalitis

Your answer is incorrect.

Explanation

Herpes simplex encephalitis causes amnesia accompanied by personality changes, focal impaired awareness seizures (previously named complex partial seizures), and the Kluver-Bucy syndrome because the virus typically enters the undersurface of the brain through the nasopharynx and preferentially attacks the temporal lobes. This condition occurs relatively frequently because herpes simplex is the most common cause of sporadically occurring (non-epidemic) viral encephalitis. In contrast, human immunodeficiency virus (HIV) encephalitis does not cause this picture.

Ref: Kaufman DM, Geyer HL, Milstein MJ, Rosengard JL. Kaufman's Clinical Neurology for Psychiatrists, 9th Edition. 2023. p. 112.

The correct answer is: Herpes simplex encephalitis

Question **37**

Not answered

Marked out of 1

Which of the following occurs in Ganer syndrome?

Select one:

- ☐ Pseudohallucinations
- ☐ Disorientation in person
- ☐ Delusions
- ☐ Clear consciousness
- ☐ Confabulation

Your answer is incorrect.

Explanation:

Ganer syndrome is characterised by approximate answers, disorientation, clouding of consciousness, hallucinations/pseudo-hallucinations motor disturbance, anxiety or apathy, normal ADLs, and sudden resolution with amnesia for the period of illness. In ICD-11, it is included under 'other specified dissociative disorders'.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 236.

Dieguez S. Ganer syndrome. Neurologic-Psychiatric Syndromes in Focus-Part II. 2018;42:1-22.

The correct answer is: Pseudohallucinations

Question **38**

Not answered

Marked out of 1

A patient with bipolar disorder and a history of mixed affective states is in the first trimester of her pregnancy. She has not been on any medications of late, but is very concerned about the possibility of relapse. Of the following, what would be the preferred medication to prescribe to her?

Select one:

- ☐ Olanzapine
- ☐ Valproate and lithium
- ☐ Lithium
- ☐ Valproate
- ☐ Lamotrigine

Your answer is incorrect.

Explanation:

For pregnant women with bipolar disorder who have had a long period without relapse, the possibility of switching to a safer drug (antipsychotic) or withdrawing treatment completely before conception and for at least the first trimester should be considered. No mood stabiliser is clearly safe. NICE recommends the use of mood stabilising antipsychotics as a preferable alternative to a mood stabiliser.

Ref: Taylor DM, Barnes TRE, Young AH. The Maudsley Prescribing Guidelines in Psychiatry, 14th Edition. 2021. p. 692.

The correct answer is: Olanzapine



Question 39

Not answered

Marked out of 1

Which one of the following medications could be used to treat pathologic crying?

Select one:

- ☐ Reboxetine
- ☐ Citalopram
- ☐ Imipramine
- ☐ Venlafaxine
- ☐ Mirtazapine

Your answer is incorrect.

Explanation:

The most conspicuous feature of pseudobulbar palsy is emotional lability. With little or no provocation, patients tend to burst into tears or, less often, laughter, with no apparent change in their mood. These unprovoked emotional expressions are referred to as pathologic laughing and crying, pseudobulbar affect, or involuntary emotional expression disorder. Amitriptyline, SSRIs (such as citalopram), or a combination of dextromethorphan and quinidine may suppress this.

Ref: Kaufman DM, Geyer HL, Milstein MJ, Rosengard JL. Kaufman's Clinical Neurology for Psychiatrists, 9th Edition. 2023. p. 39.

The correct answer is: Citalopram

Question 40

Not answered

Marked out of 1

A pregnant bipolar patient has no other treatment option but to continue valproate. Which of the following should she take in addition?

Select one:

- ☐ Vitamin K
- ☐ Thiamine
- ☐ Pyridoxine
- ☐ Folate
- ☐ Iron supplements

Your answer is incorrect.

Explanation:

Valproate is an established human teratogen. Usually, valproate should be reserved for post-menopausal women only. Its use in younger women should be treatment of last resort. There is no evidence that folate protects against anticonvulsant-induced neural tube defects if given during pregnancy, but may do so if given prior to conception (the neural tube is essentially formed by 8 weeks of pregnancy before many women realise they are pregnant). However, folate supplementation may be beneficial for early neurodevelopment and so should always be offered.

Ref: Taylor DM, Barnes TRE, Young AH. The Maudsley Prescribing Guidelines in Psychiatry, 14th Edition. 2021. pp. 260, 680, 691.

The correct answer is: Folate



Question **41**

Not answered

Marked out of 1

What would be the most appropriate drug for a depressed lady who is pregnant?

Select one:

- ☐ Sertraline
- ☐ Fluoxetine
- ☐ Paroxetine
- ☐ Citalopram
- ☐ Venlafaxine

Your answer is incorrect.

Fluoxetine has the most evidence and is safer than paroxetine. Other drugs recommended by Maudsley guidelines include Amitriptyline, Nortriptyline and Imipramine. (Maudsley guidelines- 10th edition pg 354)

The correct answer is: Fluoxetine



Question 42

Not answered

Marked out of 1

Which of the following favours psychogenic nonepileptic seizures rather than epileptic seizures?

Select one:

- ☐ Opened eyes during seizures
- ☐ Presence of automatisms
- ☐ Total duration less than 30 secs
- ☐ Predictable anatomical progression of seizures
- ☐ Side-to-side movements of the head

Your answer is incorrect.

Explanation:

Ictal eye closure has been assessed in several controlled studies and is significantly more frequent in psychogenic nonepileptic seizures (PNES) than epileptic seizures (ES). Closed eyes during an attack is considered a discriminator between PNES and ES. The mean duration of PNES is longer than ES. There is considerable evidence to suggest that a duration of more than 2 minutes is highly suggestive of PNES, although this is an arbitrary limit. Duration of longer than 10 minutes strongly suggests PNES. In studies that have compared convulsive PNES with generalised tonic clonic seizures, the proportion of events with side-to-side head or body movement, especially with coordinated agonist and antagonist activity, are significantly higher in PNES.

Signs & symptoms favouring psychogenic nonepileptic seizures (PNES)	Signs & symptoms favouring epileptic seizures (ES)
Duration longer than 10 minutes	
Vocalisation during or after seizures, complex, with affective content	Vocalisation at the beginning, primitive, has no emotional expression
Ictal crying	
Occurrence from preictal pseudosleep	Occurrence from sleep
Ictal eye closure	
Side-to-side head or body movement (compared with generalised tonic clonic seizures only)	Postictal stertorous breathing (observed in generalised tonic clonic seizures only)
Pelvic thrusting (frontal lobe partial seizures excluded)	
Rapid regaining of alertness and orientation	Post ictal confusion
Fluctuating course	
Asynchronous jerks (frontal lobe partial seizures excluded)	
Memory recall	

Adapted from Xiang et al. 2019

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Ref: Xiang X et al. Differential diagnosis between epileptic seizures and psychogenic nonepileptic seizures based on semiology. Acta Epileptologica. 2019 Dec;1(1):1-5.

The correct answer is: Side-to-side movements of the head

Question 43

Not answered

Marked out of 1

Idiopathic progressive calcium deposition in the basal ganglia is seen in

Select one:

- ☐ Wilson disease
- ☐ Meige syndrome
- ☐ Herpes simplex encephalitis
- ☐ Fahr disease
- ☐ Huntington disease

Your answer is incorrect.

Explanation:

Fahr disease, also known as idiopathic basal ganglia calcification, is a rare autosomal dominant neurodegenerative disorder, characterised by bilateral calcifications in multiple basal ganglia. Patients with Fahr disease may be asymptomatic or present with movement disorders such as parkinsonism, psychiatric disorders such as psychosis and depression, cognitive impairment, dementia, and a variety of other symptoms. The age on onset is mostly between 40 and 50 years, but the symptoms can manifest at any age. Patients who become symptomatic early in adulthood mostly develop psychiatric or cognitive disorders, like psychosis, whereas patients who become symptomatic later in life develop mainly movement disorders in combination with other clinical features.

Ref: Peters ME et al. Mechanisms of calcification in Fahr disease and exposure of potential therapeutic targets. *Neurology: Clinical Practice*. 2020 Oct 1;10(5):449-57.

The correct answer is: Fahr disease

Question **44**

Not answered

Marked out of 1

Presence of which of the following should prompt investigations for a medical cause of recurrent anxiety?

Select one:

- ☐ Family history of anxiety disorders
- ☐ Poor response to anxiolytic agents
- ☐ Onset of symptoms before the age of 25 years
- ☐ Significant anticipatory anxiety
- ☐ Presence of avoidance behaviour

Your answer is incorrect.

Explanation:

Once effective treatment has been adequately delivered and adhered to, there will be a proportion of patients with anxiety that have not remitted and a smaller proportion that remain quite symptomatic. At this point, it should be considered whether there are exogenous factors that are aggravating and/or maintaining anxiety, whether there is a medical condition, and whether the diagnosis may be wrong or incomplete (i.e. whether there is a comorbidity present that is accounting for the poor response to treatment).

Ref: Roy-Byrne P. Treatment-refractory anxiety; definition, risk factors, and treatment challenges. Dialogues in clinical neuroscience. 2015 Jun;17(2):191.

The correct answer is: Poor response to anxiolytic agents



Question **45**

Not answered

Marked out of 1

What percentage of women with a history of schizophrenia will experience a relapse of psychosis within the first 12 months postpartum?

Select one:

- ☐ 36%
- ☐ 46%
- ☐ 6%
- ☐ 16%
- ☐ 26%

Your answer is incorrect.

Explanation:

Relapse of psychosis within the first 12 months postpartum occurs in approximately 16% of women with a history of schizophrenia. This is less common and manifests differently than in women with bipolar disorder.

Ref: Meltzer-Brody S et al. Postpartum psychiatric disorders. Nature Reviews Disease Primers. 2018 Apr 26;4(1):1-8.

The correct answer is: 16%



Question **46**

Not answered

Marked out of 1

Which of the following symptoms do you see more often in the later rather than the early stages of HIV dementia?

Select one:

- ☐ Ataxia
- ☐ Paraparesis
- ☐ Depression
- ☐ Apathy
- ☐ Agitation

Your answer is incorrect.

Except paraparesis, other symptoms are seen even in early stages of HIV dementia.

The correct answer is: Paraparesis



Question **47**

Not answered

Marked out of 1

The most common psychiatric symptom in Cushing syndrome is

Select one:

- ☐ Anxiety
- ☐ Depression
- ☐ Mania
- ☐ Memory loss
- ☐ Psychosis

Your answer is incorrect.

Explanation:

Cushing syndrome is a relatively rare but serious condition that results from chronic hypercortisolism. Depression is the most prevalent and significant psychiatric disorder among patients with Cushing syndrome.

Ref: Lin TY et al. Psychiatric symptoms in Cushing's syndrome: a systematic review. Innovations in Clinical Neuroscience. 2020 Jan 1;17(1-3):30.

The correct answer is: Depression

Question 48

Not answered

Marked out of 1

Which of the following MRI findings is characteristically seen in Huntington disease?

Select one:

- ☐ Cerebellar atrophy
- ☐ Pulvinar infarct
- ☐ Knife blade gyri
- ☐ Multiple white matter hyperintensities
- ☐ Caudate atrophy

Your answer is incorrect.

Explanation:

Huntington disease is a rare neurodegenerative disorder of the central nervous system, with a genetic autosomal dominant inheritance, that first involves the basal ganglia (caudate nucleus and putamen) and results from expansion of a CAG trinucleotide repeat in the HTT (huntingtin) gene. In Huntington disease, CT/MRI show atrophy of the basal ganglia (caudate and putamen) with 'boxing' of the caudate and dilatation of the ventricles.

Ref: Bachoud-Lévi AC et al. International Guidelines for the treatment of Huntington's Disease. Frontiers in neurology. 2019;10:710.

Samuel D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 166.

The correct answer is: Caudate atrophy

Question 49

Not answered

Marked out of 1

Mr. Y has been treated for multiple sclerosis, and he developed depression following treatment. Which of the following drugs is most likely to have induced the depression?

Select one:

- ☐ Glatiramer acetate
- ☐ Tizanidine
- ☐ Amantadine
- ☐ Baclofen
- ☐ Beta interferon

Your answer is incorrect.

Beta and alpha interferons used in the treatment of multiple sclerosis is associated with depression. The role of beta interferons in the etiology of Multiple Sclerosis related depression is unclear. Standard care for initiation of beta interferon should include assessment for depression and for those with a past history of depressive illness, prophylactic treatment with an antidepressant is recommended (Ref: The Maudsley prescribing guidelines; 10th edition; pg 410)

The correct answer is: Beta interferon

Question 50

Not answered

Marked out of 1

A 45-year-old man with diabetes is brought to the emergency department with nausea, vomiting, lethargy, and muscle cramps. He has been diagnosed with depression and recently started on fluoxetine. What is the most likely metabolic complication?

Select one:

- ☐ Hypokalaemia
- ☐ Hyperkalaemia
- ☐ Hypocalcaemia
- ☐ Hyponatraemia
- ☐ Hypercalcaemia

Your answer is incorrect.

Explanation:

Most antidepressants have been associated with hyponatraemia. The most likely mechanism of this adverse effect is the syndrome of inappropriate secretion of antidiuretic hormone (SIADH). There is a strong level of supporting evidence that SSRIs (including fluoxetine) are associated with a relatively high risk of hyponatraemia. All patients taking antidepressants should be informed of and observed for signs of hyponatraemia (dizziness, nausea, lethargy, confusion, cramps, seizures).

Ref: Taylor DM, Barnes TRE, Young AH. The Maudsley Prescribing Guidelines in Psychiatry, 14th Edition. 2021. pp. 393-394.

The correct answer is: Hyponatraemia

Question **51**

Not answered

Marked out of 1

Which of the following is an effective treatment for post-stroke depression based on the best available evidence?

Select one:

- ☐ mirtazapine
- ☐ amitriptyline
- ☐ duloxetine
- ☐ fluoxetine
- ☐ venlafaxine

Your answer is incorrect.

A metaanalysis that pooled 385 patients from 6 trials published through December 2009 that evaluated the prophylactic efficacy of fluoxetine in poststroke patients. They found that fluoxetine reduced the incidence of poststroke depression, helped recovery in neurologic function, and improved independence in activities of daily living.

The correct answer is: fluoxetine

Question 52

Not answered

Marked out of 1

What rating scale would you recommend to a health visitor who wants to screen for depression in women after delivery?

Select one:

- ☐ Morgan-Russel scale
- ☐ MADRS
- ☐ Zung depression scale
- ☐ BDI
- ☐ Edinburgh Depression Scale

Your answer is incorrect.

EPDS is a 10-item self-report scale to screen for Postnatal Depression in the community. The scale can be completed in about 5 minutes and has a simple method of scoring that can be used by health visitors and midwives. Excerpted from Cox et al. Detection of postnatal depression. Development of the 10-item Edinburgh Postnatal Depression Scale. The British Journal of Psychiatry. 1987; 150: 782-786.

The correct answer is: Edinburgh Depression Scale

Question 53

Not answered

Marked out of 1

In ICD 10 which of the following is included under dissociative disorders?

Select one:

- ☐ Neurasthenia
- ☐ Somatization disorder
- ☐ Possession disorder
- ☐ Hypochondriasis
- ☐ Malingering

Your answer is incorrect.

.'Trance and possession' disorders are placed under the general rubric of dissociative disorder (ICD-10) implying a psychological causation of symptoms. Possession is characterized by a transient alteration in identity whereby one's normal identity is temporarily replaced or possessed by a spirit, ghost, deity, etc. This disorder may be culture-bound for some non-Western cultures.

The correct answer is: Possession disorder

Question **54**

Not answered

Marked out of 1

What is the case fatality rate in patients with herpes simplex encephalitis?

Select one:

- ☐ 70%
- ☐ 30%
- ☐ 10%
- ☐ 20%
- ☐ 50%

Your answer is incorrect.

Untreated, there is a case fatality of ~70% for Herpes simplex encephalitis. Treatment with intravenous aciclovir reduces fatality to 20-30%. Treatment in proven cases should continue for at least 14 days, longer in immunocompromised patients

The correct answer is: 70%



Question 55

Not answered

Marked out of 1

During the psychiatric consultation, Mr. Y says 'I feel like a woman trapped in a man's body'. This describes a core clinical feature seen in

Select one:

- ☐ Schizophrenia
- ☐ Female to male transvestites
- ☐ Female to male transsexuals
- ☐ Male to female transvestites
- ☐ Male to female transsexuals

Your answer is incorrect.

Transsexualism is characterized by the feeling of one's gender being different from one's biological sex. "These patients have an enduring belief that they are really of the opposite sex. This is associated with marked unhappiness and discomfort in the birth sex and a persistent desire to live and be treated as a member of the opposite sex Transvestism (the wearing of clothes of the opposite sex for sexual or other purposes, not associated with the core belief of incorrect gender found in the transsexual)

The correct answer is: Male to female transsexuals



Question **56**

Not answered

Marked out of 1

A 33 year old man has intractable seizures. Which of the following factors may support a diagnosis of non-epileptic seizures rather than true epilepsy?

Select one:

- ☐ No past psychiatric history
- ☐ Regaining alertness and orientation rapidly
- ☐ Less likely to maintain body tone during a seizure
- ☐ More likely to sustain injury from seizures
- ☐ Absence of social stressors

Your answer is incorrect.

The following factors in the history may support a diagnosis of non-epileptic seizures. (Dilley and Fleminger, 2006) 1. A psychiatric history 2. Regaining alertness and orientation rapidly 3. Ability to recall events clearly 4. The presence of social stressors.

The correct answer is: Regaining alertness and orientation rapidly

Question **57**

Not answered

Marked out of 1

As regards drug therapy in palliative care all are true except

Select one:

- ☐ Amitriptyline is useful for neuropathic pain
- ☐ SSRIs have well established evidence base for depression in palliative care
- ☐ Tricyclics may produce troublesome side effects and delirium.
- ☐ Lorazepam is useful for acute anxiety
- ☐ Risperidone may be useful for psychotic symptoms in terminally ill patients

Your answer is incorrect.

The evidence base for treating various psychiatric symptoms in palliative care is feeble; the use of SSRIs is empirical rather than evidence-based.

The correct answer is: SSRIs have well established evidence base for depression in palliative care



Question **58**

Not answered

Marked out of 1

MRI shows hypointensity of striatum in patients with

Select one:

- ☐ HIV Dementia
- ☐ Meige syndrome
- ☐ Wilson's disease
- ☐ Herpes simplex encephalitis
- ☐ Fahr's disease

Your answer is incorrect.

The commonest neurological features of FD are Parkinsonism, chorea, dystonia, tremor, gait disturbance, dysarthria, seizures, and myoclonus. Fahr's syndrome - specific causes of calcium deposition in the basal ganglia such as hypoparathyroidism. Radiological basal ganglia calcification without clinical features on CT may occur at a rate of about 0.9% of cases. MRI shows hypointensity of the striatum. EEG is non-specific - may show diffuse changes

The correct answer is: Fahr's disease

Question 59

Not answered

Marked out of 1

'Baby blues' are reported in what proportion of new mothers?

Select one:

- ☐ 1 in 10
- ☐ 3 in 8
- ☐ 1 in 5
- ☐ 3 in 4
- ☐ 2 in 5

Your answer is incorrect.

Baby blues: Up to 3/4 of new mothers will experience a short-lived period of tearfulness and emotional lability starting two or three days after birth and lasting one to two days. This is common enough to be easily recognizable by midwifery staff and requires only reassurance and observation towards resolution. There is weak evidence that it may be related to post-partum reductions in levels of oestrogen, progesterone, and prolactin (which do occur around 72 hrs after the birth). Ref: Oxford handbook of psychiatry; 756

The correct answer is: 3 in 4



Question 60

Not answered

Marked out of 1

Doctor-shopping is an entity associated with which psychiatric condition?

Select one:

- ☐ Depression
- ☐ Munchausen syndrome
- ☐ Hypochondriasis
- ☐ Somatisation disorder
- ☐ Conversion disorder

Your answer is incorrect.

Somatisation disorder: "Diagnosis is usually only suspected after negative findings begin to emerge as normal medical practice is to take complaints at face value. Key diagnostic feature is multiple, atypical, and inconsistent medically unexplained symptoms in a patient under the age of 40. There is excessive use of medical service and alternative therapies. Chronic cases will have had large numbers of diagnostic procedures and surgical or medical treatments. High risk of iatrogenic harm and iatrogenic substance dependence. Hostility and frustration can be felt on both sides of the doctor-patient relationship. There may be doctor-shopping and splitting of the attitudes of staff caring for them. Psychological approaches to treatment are hampered by on-going investigations of ever rarer diagnostic possibilities and by the attribution of symptoms to fictitious but named medical entities". (Excerpt from Oxford Handbook of Psychiatry; 744)

The correct answer is: Somatisation disorder

Question **61**

Not answered

Marked out of 1

Which of the following statements is incorrect about somatisation disorder?

Select one:

- ☐ Most frequent symptoms are non specific and atypical
- ☐ It is associated with significant psychological distress and functional impairment
- ☐ Usually presents before the age of 40
- ☐ Symptoms are usually localised to a single physiological system
- ☐ Usually have long complex medical histories

Your answer is incorrect.

Somatisation disorder is a chronic disorder of multiple medically unexplained symptoms, affecting multiple organ systems presenting before the age of 40. It is associated with significant psychological distress, functional impairment, and repeated presentations to medical services. Full blown somatisation disorder or Briquet's syndrome probably represents the severe end of a continuum of abnormal illness behaviour (Excerpt from Oxford Handbook of Psychiatry). Patients will have long, complex medical histories, although at interview may minimize all but the most recent symptomatology.

The correct answer is: Symptoms are usually localised to a single physiological system

Question **62**

Not answered

Marked out of 1

What percentage of patients will die by completed suicide in the 12 months after an initial parasuicidal act?

Select one:

- ☐ 0.02
- ☐ 0.15
- ☐ 0.05
- ☐ 0.1
- ☐ 0.01

Your answer is incorrect.

Risk of suicide within one year of DSH - 0.7% (nearly 1 in 100); more in males 1.1%; 0.5% in females. This is 66 x the general population risk. Roughly 1% will die by completed suicide in the 24 months after the initial parasuicidal act, with the risk highest in the weeks following the original act. This represents mortality by suicide 50-100 times that of the general population. Ref: Oxford handbook of psychiatry; 730

The correct answer is: 0.01

Question **63**

Not answered

Marked out of 1

The risk factors for completed suicide would include all except

Select one:

- ☐ Female sex
- ☐ Living alone
- ☐ Widowed
- ☐ Unemployed
- ☐ Elderly

Your answer is incorrect.

Risk factors for completed suicide Socio-demographic factors: Male sex, Elderly, Single, divorced, or widowed, Living alone, poor social support, Unemployed or low socio-economic class Personal/mental health factors: Previous Parasuicide or DSH, Any mental disorder (greatest risk in major depression and anorexia nervosa, then functional psychosis, then neurotic and personality disorders), Dependence on alcohol or drugs, Recent inpatient psychiatric treatment, Concurrent physical disorder and Recent bereavement

The correct answer is: Female sex



Question **64**

Not answered

Marked out of 1

Presentations featured by 'episodic panic attacks' are reported most often in patients with;

Select one:

- ☐ Hypothyroidism
- ☐ Addison's disease
- ☐ Cushing's syndrome
- ☐ Acromegaly
- ☐ Pheochromocytoma

Your answer is incorrect.

Pheochromocytoma: Catecholamine-secreting tumour. Physically - tachycardia, labile hypertension, headache, sweating, and palpitations - episodic. May mimic panic attacks. Screening by testing for urinary catecholamines (Vanillyl mandelic acid, metanephrines). The best diagnostic test is a plasma metanephrine level which is more specific.

The correct answer is: Pheochromocytoma

Question **65**

Not answered

Marked out of 1

The peak incidence of head injury is between the ages of

Select one:

- ☐ 25-34 Years
- ☐ 54-65 Years
- ☐ 45-54 Years
- ☐ 15-24 Years
- ☐ 35-44 Years

Your answer is incorrect.

Peak incidence of head injury is between the ages of 15 - 24 years. Concussion damage causes transient coma for hours followed by apparent complete clinical recovery. Brain contusion leads to prolonged coma, focal signs and lasting brain damage.

The correct answer is: 15-24 Years



Question **66**

Not answered

Marked out of 1

The most common co-morbid psychiatric condition associated with hypochondriasis is

Select one:

- ☐ Depression
- ☐ Generalized anxiety disorder
- ☐ Conversion disorder
- ☐ OCD
- ☐ Panic disorder

Your answer is incorrect.

Patients with hypochondriasis have a high rate of psychiatric comorbidity. In one outpatient clinical series, 88% of patients with hypochondriasis had one or more concurrent psychiatric disorders, the most common being generalized anxiety disorder (71%), dysthymic disorder (45.2%), major depression (42.9%), somatization disorder (21.4%), and panic disorder (16.7%). These patients are 3 times more likely to have a personality disorder than the general population. Arch Gen Psychiatry. Feb 1992;49(2):101-8.

The correct answer is: Generalized anxiety disorder

Question **67**

Not answered

Marked out of 1

Which lobe of the brain is commonly affected in patients with herpes simplex encephalitis?

Select one:

- ☐ Frontal
- ☐ Parietal
- ☐ Occipital
- ☐ Temporal
- ☐ Cerebellar

Your answer is incorrect.

NULL

The correct answer is: Temporal



Question **68**

Not answered

Marked out of 1

Mrs. P suffers from epilepsy and clinical depression. She has been tried on SSRIs with no benefit and is keen to try older antidepressants. The safest choice in her case would be;

Select one:

- ☐ Amitriptyline
- ☐ Moclobemide
- ☐ Dothiepin
- ☐ Bupropion
- ☐ Clomipramine

Your answer is incorrect.

Moclobemide is not known to be pro-convulsive and is a good choice. SSRIs are considered to be a good choice too in epileptic patients. Most TCAs are epileptogenic and ideally should be avoided completely. Amitriptyline, Dothiepin, Clomipramine and Bupropion should be avoided. Use mirtazapine, Venlafaxine, Reboxetine and duloxetine with care in epileptic subjects. Ref: Maudsley prescribing guidelines, 10th edition; 337

The correct answer is: Moclobemide

Question 69

Not answered

Marked out of 1

Which one among the following is not an observation normally associated with Herpes simplex encephalitis?

Select one:

- ☐ Magnetic resonance imaging (MRI) is the imaging of choice in HSE
- ☐ Gold standard for in vivo diagnosis is CSF polymerase chain reaction (PCR) for herpes viruses.
- ☐ Cerebrospinal fluid (CSF) examination commonly reveals a lymphocytosis and a raised protein
- ☐ The EEG is invariably abnormal in HSE.
- ☐ CT Scan usually reveals signal change and swelling within the frontal lobes

Your answer is incorrect.

Investigatory findings observed in patients with Herpes simplex encephalitis: Neuroimaging usually reveals signal change and swelling within the temporal lobes; this is often visible on CT, but is more easily seen using MRI. Significant brain swelling may lead to raised intracranial pressure. Cerebrospinal fluid (CSF) examination commonly reveals a lymphocytosis and raised protein, Gold standard for in vivo diagnosis is CSF polymerase chain reaction (PCR) for herpes viruses, which has a sensitivity and specificity of ~95%. Magnetic resonance imaging (MRI) provides the most sensitive method of detecting early lesions and is the imaging of choice in HSE. The EEG is invariably abnormal in HSE. "Non-specific slowing may be the only feature early in the illness; the later stages are more likely to be associated with high voltage periodic lateralising epileptiform discharges"(Retrieved from <http://jnnp.bmj.com/content/73/3/237.full.pdf>). If present they are diagnostically useful but they are not specific for HSE.

The correct answer is: CT Scan usually reveals signal change and swelling within the frontal lobes

Question 70

Not answered

Marked out of 1

Most cases of herpes simplex encephalitis are caused by

Select one:

- ☐ Herpes simplex type 1
- ☐ Human immunodeficiency virus
- ☐ Herpes simplex type 2
- ☐ Human herpes virus 6
- ☐ Human herpes virus 7

Your answer is incorrect.

In children older than 3 months and in adults, HSE is usually localized to the temporal and frontal lobes and is caused by herpes simplex virus type 1 (HSV-1). In neonates, however, brain involvement is generalized, and the usual cause is herpes simplex virus type 2 (HSV-2), which is acquired at the time of delivery (Herpes Simplex Encephalitis: HSE - Medscape Reference. Retrieved from <http://emedicine.medscape.com/article/1165183-overview>).

The correct answer is: Herpes simplex type 1



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